Week 14: the installation has moved from design into controlled build

At the conclusion of Week 14, the JPMC AI-Assisted Connected Patient Flow System reaches the operational midpoint of core development. Initial project scoping, clinical workflow analysis, target technical architecture, and clinical-safety boundaries have been formally completed. Engineering delivery is now concentrated on core application services, real-time queue dashboards, role-based identity management, immutable audit logging, document storage, and outbound notification adapters across development and test environments.

OVERALL STATUS

ON TRACK, WITH DEPENDENCIES

Delivery remains aligned to the 24–28 week baseline, conditioned on timely JPMC governance sign-offs, clinical leadership availability, and interface access.

BASELINE ARCHITECTURAL & OPERATIONAL SETUP

Area	Week 14 Position	Evidence & Operational Implication
Governance & Delivery	Scope, pilot-first SDLC, roles, and risk register locked.	Biweekly steering, weekly engineering, and clinical safety reviews active.
Operational Scope	One Outpatient Department (OPD) and one Emergency workflow selected.	Pilot configured to benchmark wait intervals, triage throughput, and uptake.
Workflow & Mapping	Patient, doctor, nurse, administrative, and queue paths fully mapped.	Detailed user stories, exception matrices, and data dictionary codified.
Installation & Infra	Segregated Dev, Staging, and Production topology specified.	Formal JPMC sign-off required for hosting model, firewall rules, and endpoints.
Clinical Safety	Advisory-only AI boundaries established; diagnostic autonomy barred.	System strictly predicts queue ranges and surfaces vitals flags without prescribing.

Active Core Development (Weeks 8–15)

The ongoing sprint window delivers essential operational components: secure authentication, patient demographic capture, token generation, live waiting-room queues, and provider consultation consoles. Predictive waiting-time models and vital-sign trend alerts remain strictly behind feature toggles to guarantee full continuity should algorithmic services experience disruption.

Installation Controls in Force

- **Access control:** Named administrative accounts, mandatory MFA, and role-based segregation enforced across all staging environments.
- **Integration staging:** Live hospital networks remain completely isolated; all API adapters run against synthetic payloads.
- **Clinical sign-off:** Department heads must formally review and approve consultation screens prior to any rehearsal.
- **Production barrier:** Zero live patient records will be ingested until disaster recovery, device audits, and training protocols are certified.

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Source: JPMC Implementation & SDLC Proposal v1.0

The next four weeks convert the build into testable evidence

The project timeline now transitions from isolated feature development into integrated release verification. Over Weeks 15 through 19, engineering deliverables will be converted into verifiable operational evidence through end-to-end integration testing, system rehearsals, security certifications, and clinical user acceptance testing (UAT) conducted directly with nominated JPMC operational stakeholders.

DELIVERY SCHEDULE & SEQUENTIAL GATES

Window	Primary Work Stream	Required Gate Output & Ownership
Week 15	Close core development baseline; freeze APIs, event queues, and UI.	Demonstrable build with release notes and defect backlog (Dev Lead / Arch).
Weeks 16–17	Execute system integration, performance load, security, and AI bounds tests.	Verified test matrix mapped to requirements traceability sheet (QA / Security).
Weeks 18–19	Conduct structured UAT with JPMC staff; complete downtime drills.	Formal UAT sign-off; 0 critical defects, resolved severity-2 items (JPMC Leads).
Weeks 20–23	Execute controlled pilot in pilot OPD and Emergency departments.	Go/no-go audit based on uptime, adoption rate, and wait metrics (Steering Com.).
Weeks 24–28	Phased production cutover, operational hypercare, and final handover.	Production readiness approval and full operational transfer (JPMC Exec).

CRITICAL JPMC DEPENDENCIES

- **Stakeholders:** Finalize named clinic heads and nursing leads for UAT validation sessions.
- **API Access:** Grant sandbox credentials for HIS, billing, pharmacy, and laboratory interfaces.
- **Infrastructure:** Approve on-premise virtual machine sizing, network VLANs, and firewall rules.
- **Hardware:** Confirm deployment schedule for display monitors, tablets, and receipt printers.

ACTIVE RISK & MITIGATION REGISTER

Risk Event	Mitigation Protocol	Lead Owner
HIS interface delay	Synthetic sandbox simulation & manual batch fallback.	JPMC IT / HIS Lead
Clinical workflow friction	Co-designed UI & hands-on scenario drills.	Clinical Safety Lead
Power & network instability	Offline caching & physical paper ticket protocol.	Facilities & IT Ops
Out-of-scope expansion	Strict change control governing baseline features.	Project Steering Com.

IMPLEMENTATION OWNER RECOMMENDATION

Authorize progression into Week 15 code freeze and approve entry into Week 16 integration testing, contingent upon JPMC confirming integration sandbox access, device delivery schedules, and designated clinical evaluators. The system must be judged on workflow resilience, verifiable safety margins, and clinical user adoption rather than algorithmic novelty.

Prepared by: JPMC Report · Implementation Owner, AI-Assisted System Installation · Reporting point: Week 14

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